

Tranexamic Acid (TXA) Guideline

History

- What was the mechanism of injury – blunt (MVC, fall, blow to body vs. penetrating (stabbing, GSW, foreign body)?
- Other medical conditions? Did a medical condition contribute to the mechanism of injury?
- Medications – Coumadin? Plavix? Aspirin? Pradaxa? Xarelto? Eliquis? (any blood thinners or anticoagulants)

Key Concepts

- TXA is now being used to treat severely injured trauma patients who have or are at risk for severe hemorrhage

MARCHES Protocol

- **Massive bleeding control** - tourniquet, wound packing
- **Airway** – NPA/OPA/ Cric
- **Respiratory** – decompress chest if tension pneumothorax suspected, occlusive dressing for open pneumothoraces
- **Circulation**- IV/IO, pelvic binder, reassess tourniquet/wound packing
- **Hypothermia care**/Hike vs Helicopter (consider evac options)
- **Eye injuries** – cover with rigid shield and no pressure on the eye
- **Spinal motion restriction** if indicated

Trauma Criteria

Adult trauma (≥18 years) patients at risk for significant hemorrhage from blunt or penetrating trauma within last 3 hours with:

- central trunk abdomen or pelvis wounds located from mandible to mid-thigh OR
- major amputations proximal to the wrist or ankle with uncontrolled bleeding OR
- suspected severe internal injuries from blunt or penetrating trauma

EMT

- Follow Trauma General Patient Care Guideline
- Follow appropriate Trauma Guideline

EMT/AEMT - Expanded Scope

- **TXA** via IO per guidelines in Paramedic section
- For airway bleeding or epistaxis see Epistaxis Guideline

Paramedic

For Patients in HEMORRHAGIC SHOCK:

To administer: TXA trauma patients must meet above Trauma Criteria and have signs of acute hemorrhagic shock as evidenced by:

- Systolic Blood Pressure < 70 mmHg **OR**
- Systolic Blood Pressure < 90 mmHg with Heart rate ≥ 110 beats per min **OR**
- ETCO₂ < 25 **OR**
- Age ≥ 65 yo and SBP ≤ 100 **AND** HR ≥ 100 beats per minute
- **Tranexamic (TXA)** 2g IV/IO slow push

Contraindications:

- TXA shall not be used for isolated head injuries
- TXA shall not be used for hemorrhagic shock from GI bleeding